

PSYCHOSOCIAL AND WHS RISK REPORT — SWITCHBOARD SERVICES, LOGAN HOSPITAL

Framework · Hazard Matrix · Existing Controls · Proposed Controls — applied together

Cory Lea Shepherd, AO3 Switchboard Services · August 2026 · Prepared under the Work Health and Safety Act 2011 (Qld), the WHS Regulation 2011 (Qld), the Managing the risk of psychosocial hazards at work Code of Practice 2022, and consistent with ISO 45003. Offered as a worker's contribution under ss 47–49 and for the clause 7.2 assessment. Every hazard is described as a feature of the work and its systems; the 'existing controls' column records the position as known to me as an operator of the room; nothing in this report is an allegation against any person.

PART 1 — FRAMEWORK

Section 19 of the Act requires the health of workers — expressly including psychological health — to be ensured so far as reasonably practicable. The Regulation requires psychosocial hazards to be identified, the risks assessed, controls implemented in the order of the hierarchy, and controls reviewed, in consultation with affected workers (ss 47–49). The Code supplies the recognised hazard categories, the assessment factors (duration, frequency and severity of exposure), and the data sources. Risk ratings below apply the Code's factors; where a category of harm has already materialised in a diagnosed psychological injury within the room, the consequence limb is treated as established rather than speculative.

PART 2 — THE HAZARD MATRIX (framework applied)

#	HAZARD (Code)	HOW IT PRESENTS	EXISTING CONTROLS IDENTIFIED	REQUIRED BY	RISK (duration × frequency × severity)	PROPOSED CONTROL
1	High job demands	200–300 calls/shift, many distressed or urgent; multiple concurrent systems; emergency codes throughout; absorbed functions when support absent	None identified: no demand-based staffing standard, no surge protocol, no occupancy bound	s 19; Reg (identify/assess/control); Code — job demands	HIGH — sustained (years) × daily × injury of this kind has occurred	Evidence-based rostering (P7); escalation/backup pathways (P1, P4); occupancy standard (methodology (e))
2	Low job control	Operational changes effective without operator consultation; no operator input to roster design	None identified: no consultation step precedes operational change	ss 47–49 (consultation is mandatory, not optional); Code	HIGH — the Code's high-risk demands/control combination, sustained	Consultation before change (P2); real-time team consultation in supervisor decisions (P6); roster input (P7)
3	Poor support	Supervision and decision-support not reliably available in real time; after-hours and weekend work continues unchanged	None identified after hours: no on-shift supervision structure; business-hours management only	s 19; Code — support	HIGH — daily after-hours exposure, years	Integrated manager in hours (P3); 24/7 rotating on-shift supervisor (P6); structured handover (P8)
4	Low role clarity	Functions accrete without documented boundaries (complaints, departmental support, roster administration)	None identified: no documented boundary or escalation map for accreted functions	Code — role clarity	MEDIUM-HIGH — sustained; drives blame absorption	Documented boundaries + escalation (P1, P4); decision log/handover (P8)
5	Poor organisational change management	Process and access changes made without consulting affected operators; effects discovered live	None identified	Code — change management; ss 47–49	MEDIUM-HIGH	Consultation before change, systematised (P2)
6	Inadequate reward and recognition	No development pathway within the room; competence and responsibility unrecognised	None identified: no progression or recognition structure exists for the room	Code — reward and recognition	MEDIUM — sustained; retention risk	Rotating supervisor function with in-charge recognition and trained pathway (P6)
7	Poor organisational justice	Complaints and workplace matters raised without acknowledgment, outcome or feedback; inconsistent process standards	None identified: no response standard; cl 7.1.11 feedback not practised	Code — organisational justice; cl 7.1.11 (agreement)	HIGH — sustained; injury of this kind has occurred	Complaint-response standard (P4); one rulebook (P5); written feedback per matter

#	HAZARD (Code)	HOW IT PRESENTS	EXISTING CONTROLS IDENTIFIED	REQUIRED BY	RISK (duration × frequency × severity)	PROPOSED CONTROL
8	Remote/isolated work (after hours)	Nights/weekends worked without present supervision while all functions continue	None identified	Code — remote/isolated work	HIGH after-hours	24/7 on-shift supervisor (P6); handover continuity (P8)
9	Distressed and aggressive callers	First point of contact; a substantial proportion of contacts in distress, fear, conflict or crisis; accusations directed at the operator	None identified: no debrief/defusing step, no post-call recovery arrangement	s 19; Code — violence/aggression (verbal)	HIGH — daily, cumulative	Triage end-point + escalation (P1, P4); debrief and recovery arrangements (methodology (a), (b))
10	Fatigue	Rostering capable of sub-minimum breaks; team-wide pattern of fatigue absorbed through personal leave on/after high-volume periods — visible in rosters read against leave and demand data	Informal only: workers' own personal leave operating as the de facto fatigue control	Award/FRMS minimum breaks; s 19; Code	HIGH — a sub-minimum break has occurred and been found by the Regulator's reviewing officer to be unreasonable management action	Fatigue rules built into roster design — recovery provided by the roster, not purchased by the worker (P7)

PART 3 — WHAT THE MATRIX DISCLOSES

Read as a whole, the matrix records a single consistent position: each identified hazard has been operating without documented identification, without assessment, without controls selected through the hierarchy, and without the consultation sections 47–49 require — over a sustained period, in a room performing a patient-safety function, in which a psychological injury of the kind these hazards produce has already occurred. The only control identified anywhere in the matrix is informal and worker-funded (personal leave operating as the de facto fatigue control). This is recorded factually, from the operator's vantage point, and is exactly the question the clause 7.2 assessment exists to examine: the assessment can confirm, correct, or complete this matrix — and I ask that it do so with the operators consulted.

PART 4 — THE PROPOSED CONTROLS, INTEGRATED

P1 Patient-safety design (ownership and backup for on-call/paging accuracy; workable directory pathway; defined escalation) · P2 Consultation before operational change · P3 The rotational integrated manager (in hours, on the phones, in the room) · P4 Complaint-response standard (1 business day acknowledgment / 3 days response / automatic escalation) · P5 One rulebook for the room · P6 The 24/7 rotating on-shift supervisor — authority follows the clock; real-time decisions consulted with the room; recognition and a trained pathway · P7 Evidence-based rostering — coverage from the demand data, fatigue rules built into design, equity visible · P8 Structured handover and decision log. Implementation is from existing resources except the P6 allowance; each control maps to the matrix rows above; controls to be selected and sequenced in consultation, with owners, dates and review.

PART 5 — CONSULTATION

This report is an input to the assessment, offered under ss 47–49, to be read with the companion Assessment Methodology proposal. I will attend any discussion at any time with a support person and my union representative, and I ask that the scoping discussion address each matrix row and record the outcome.

Cory Lea Shepherd · AO3, Switchboard Services, Logan Hospital · 0417 400 227 · coryshepherd1@hotmail.com